

9-7 DISCHARGE SUMMARY

Patient Name: Marilyn Sue Stone

Patient ID: 116588 **DOB:** January 24, ---- **Age:** 50 **Sex:** F

Date of Admission: 7 February ----

Date of Discharge: 10 February ----

Admitting Physician: Jesse D. Smith, MD, Orthopedic Surgery

Procedure Performed: Right total hip replacement

Complications: None

Discharge Diagnosis: Failed total hip

Chief Complaint: Pain in right hip

HISTORY OF PRESENT ILLNESS: This is a 50-year-old female with a long history of bilateral hip disease. She was born with arthrogryposis. She has a right total hip replacement done about 28 years ago. She is admitted now because of X-ray evidence of loosening of the right femoral implant and pain with ambulation.

PHYSICAL EXAM: Limited to musculoskeletal system: The right leg is one quarter inch short. She has external rotation fixed at 25 degrees. Internal rotation of 0 degrees. Abduction 10 degrees. She has pain with all these ranges of motion.

DIAGNOSTIC DATA: LABORATORY: The patients preoperative laboratory evaluation was done as an outpatient. It was all within normal limits. Cultures of her hip showed no growth. Urinalysis showed no growth. Her post-operative electrolytes showed a slightly decreased sodium of 132 due to dilution. Her hemoglobin was 10.9, hematocrit 32.2 with a white count of 10,300. The pro time was 13.5 with an INR of 1.1. On the second post-operative day, her hemoglobin was 9.2, hematocrit 26.5 with a white count of 10,000 and a pro time of 24 with an INR of 2.1.

HOSPITAL COURSE: The patient was admitted to the hospital and the day after admission she underwent the right total hip replacement revision. Post-operatively she did well. Her drain was removed on the fourth post-operative day. She was transferred to the skilled nursing facility at Quali-Care Clinic for continued care and rehabilitation.

SURGICAL PROCEDURE: Revision, right total hip replacement.

COMPLICATIONS: None

(Continued)

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DISPOSITION: Patient will be followed in the SNF Unit by me and/or my P.A. My office is to be notified immediately if she exhibits fever, pain, wound dehiscence, bleeding, redness or swelling at surgical site, foul smelling discharge, or any other signs of post-op infection. Physical therapy is to be consulted on this patient as per my orders.

Jesse D. Smith, MD, Orthopedic Surgery

JDS:LAM

D: 10 February ----

T: 02/15/2018

C: Markus LeRoy Johnson, PA-C, at the physical therapy clinic